INFORMATION TO KEEP WITH YOU

# My complete treatment handbook

## Nivolumab + ipilimumab · colorectal after chemotherapy

Understand your treatment. Know what to expect. Keep your plan to hand.

**Displayed calendar**
**21 days**
See the active treatment phase

**Treatment stages**
**2**
Explained in the treatment reference

**Regimen source**
**NCCP 00551**
Version 7a

> Cycles 1–4: nivolumab 3 mg/kg, then ipilimumab 1 mg/kg, Day 1 every 21 days.

## Treatment stages

01 **Cycles 1–4: two immunotherapies**
Every 21 days

02 **Cycle 5 onwards: nivolumab alone**
Team-selected maintenance schedule

Your team confirms the active stage and dates. The full treatment guide explains each step.

## Inside your handbook


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**My name** _____

**Hospital / oncology team** _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

SACTCHECK · CHAPTER 01

# Understanding your treatment

Nivolumab + ipilimumab · colorectal after chemotherapy · National Cancer Control Programme (NCCP) 00551

## When to get help

**Emergency: call 112 / 999**

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

**Contact your oncology service immediately**

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Contact oncology immediately for new breathlessness, persistent diarrhoea, severe tummy pain, yellow skin or eyes, much less urine, spreading or blistering rash, severe headache, marked weakness, confusion, unusual thirst or passing much more urine. Severe breathing difficulty, chest pain, collapse or seizure needs emergency care. Effects can begin after treatment ends.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

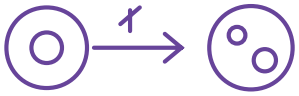
## Three medicines. Different jobs.

These two immunotherapy medicines aim to control bowel cancer with a confirmed MSI-H/dMMR deoxyribonucleic acid (the cell’s genetic material) (DNA)-repair feature. This pathway follows previous fluoropyrimidine-based combination chemotherapy.

IMMUNOTHERAPY

### Nivolumab

Opdivo



#### Blocks PD-1

Nivolumab blocks PD-1, a brake on immune T cells. This can help immune cells attack cancer, but can also inflame healthy organs.

Immune T cell → PD-1 brake blocked → Immune attack

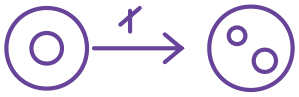
#### What to watch for

Report new breathing, bowel, hormone, skin, heart or nerve symptoms promptly.

IMMUNOTHERAPY

### Ipilimumab

Yervoy



#### Blocks CTLA-4

Ipilimumab blocks another immune brake, CTLA-4, helping activate T cells. Combining two immune medicines can increase anti-cancer activity and also the risk of serious immune inflammation.

Immune activation → CTLA-4 brake blocked → T-cell response

#### What to watch for

Report diarrhoea, abdominal pain, skin changes, headache, weakness or other new symptoms immediately.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

## Your cancer and treatment aim

A validated tumour result helps select this treatment.

#### MSI-H and dMMR

These describe a feature of the cancer's DNA-repair system. The result helps identify colorectal cancers that may respond to immunotherapy. Ask whether a genetics assessment is relevant.

**Choices and goals**

Discuss the likely benefit, uncertainty, other treatment options, clinical trials and symptom-focused care in relation to your priorities.

## How the two medicines work

They act at two different immune-control points.

**Why side effects need prompt assessment**

Immune activity may inflame healthy organs. The combination can cause serious effects, and symptoms may continue or begin after ipilimumab or all treatment stops. Pauses, steroids or permanent discontinuation may be needed.

## Evidence for this colorectal setting

CheckMate 142 followed 119 previously treated people receiving nivolumab 3 mg/kg plus ipilimumab 1 mg/kg, followed by nivolumab alone.

The four-year follow-up reported sustained responses in this single-arm cohort. Without a randomised control group, it cannot establish how much longer a person lives compared with another treatment.

SACTCHECK · CHAPTER 02

# Your schedule, tests and appointments

Nivolumab + ipilimumab · colorectal after chemotherapy · NCCP 00551

## Treatment → review → next steps

### Treatment stages

01 **Cycles 1–4: two immunotherapies**
Every 21 days

02 **Cycle 5 onwards: nivolumab alone**
Team-selected maintenance schedule

Your team confirms the active stage and dates. The full treatment guide explains each step.

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

EVERY 21 DAYS

### Cycles 1–4: two immunotherapies

C1 C2 C3 C4

Nivolumab 3 mg/kg followed by ipilimumab 1 mg/kg on Day 1. Both are given as infusions. The first nivolumab-only dose is due three weeks after the final combination dose, subject to clinical review.

TEAM-SELECTED MAINTENANCE SCHEDULE

### Cycle 5 onwards: nivolumab alone

C5 C6 C7 …

Nivolumab alone: 240 mg intravenous (IV) or 600 mg under the skin every two weeks, or 1200 mg under the skin every four weeks. Continue while helping and tolerable, until progression or unacceptable toxicity. This colorectal regimen does not set a fixed overall two-year limit.

Your team confirms each stage and discusses any changes with you.

BASELINE → TREATMENT → FOLLOW-UP

## Testing across both treatment phases

Your team books tests and reviews their results. Imaging and tumour-marker timing is individualised; no universal interval is mandated here.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00551 v7a official regimen

## Tests in your plan

### Carcinoembryonic antigen (CEA) — colorectal plans

If part of your plan

WHEN

**For a colorectal cancer plan, if your team uses CEA; your team sets the interval.**

A marker trend interpreted alongside symptoms, examination and scans.

Record the baseline, latest result and next date. A marker change alone does not establish progression.

THE WHOLE TREATMENT JOURNEY

# Nivolumab + ipilimumab · colorectal after chemotherapy: treatment and checks

### MSI-H/dMMR metastatic colorectal cancer after fluoropyrimidine-based combination chemotherapy

Cycles 1–4: nivolumab 3 mg/kg, then ipilimumab 1 mg/kg, on Day 1 every 21 days. The first nivolumab-only dose is due three weeks after the final combination dose, subject to clinical review. Nivolumab alone: 240 mg IV or 600 mg under the skin every two weeks, or 1200 mg under the skin every four weeks.

**Read from left to right.** Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

**Treatment, tests and reviews across the course**

| Care / check | Before starting | Combination cycles 1–4 | Transition to nivolumab alone | Maintenance | After treatment |
|---|---|---|---|---|---|
| **Blood count, kidneys, liver, glucose and thyroid** NCCP 00551 | Baseline results | Before every cycle | Review results and symptoms before changing phase | Before every maintenance cycle | Follow persistent abnormalities |
| **Symptoms and other hormones** NCCP 00551 | Discuss autoimmune disease, infections, medicines and earlier immune treatment | Assess immune symptoms; cortisol if indicated | A phase change does not remove immune risks | Report symptoms immediately; extra tests as indicated | Monitoring for at least five months after the last dose |

| Care / check | Before starting | Combination cycles 1–4 | Transition to nivolumab alone | Maintenance | After treatment |
|---|---|---|---|---|---|
| **Treatment and observation** NCCP 00551 | Confirm MSI-H/dMMR result and correct treatment line | Nivolumab first; then ipilimumab; vital signs and post-infusion observation | First maintenance dose three weeks after final combination | Confirm selected dose, route and two-/four-week interval | Retain the immunotherapy alert card |
| **Scans and response** NCCP 00551 | Agree assessment plan | Scans and markers as directed | Review benefit and suitability for maintenance | Review benefit, tolerability and duration | Agree next care and follow-up |

### What each check is for

**1** **Cycles 1–4: two immunotherapies**

Every 21 days

These two immunotherapy medicines aim to control bowel cancer with a confirmed MSI-H/dMMR DNA-repair feature. This pathway follows previous fluoropyrimidine-based combination chemotherapy.

**Ask / act:** Confirm the current phase, prescription, tests and support medicines.

**2** **Cycle 5 onwards: nivolumab alone**

Team-selected maintenance schedule

These two immunotherapy medicines aim to control bowel cancer with a confirmed MSI-H/dMMR DNA-repair feature. This pathway follows previous fluoropyrimidine-based combination chemotherapy.

**Ask / act:** Confirm the current phase, prescription, tests and support medicines.

### At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

## Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

### Combination appointments

During the first four cycles, nivolumab is infused first over about 30 minutes, then ipilimumab over about 30 minutes. Allow extra time for checks and observation. Nivolumab alone: 240 mg IV or 600 mg under the skin every two weeks, or 1200 mg under the skin every four weeks.

STEP 2

### Changing phase

STEP 3

### Tests and reviews

Before starting and before every cycle: blood count, kidney and liver profile, glucose and thyroid function. Cortisol is checked if clinically indicated. Disease assessment follows the consultant plan. Continue monitoring for late immune effects for at least five months after the final dose.

STEP 4

### Alert card and home care

There is no take-home chemotherapy pump. Keep the card, medicine list, symptom record and urgent contact numbers accessible.

STEP 5

### Treatment duration

Continue while helping and tolerable, until progression or unacceptable toxicity. This colorectal regimen does not set a fixed overall two-year limit.

## Combination first, then nivolumab alone

Keep both phases in your written plan.

### Four combination cycles

Nivolumab 3 mg/kg and ipilimumab 1 mg/kg are given on Day 1 every 21 days for four cycles. Nivolumab is infused first, then ipilimumab; each infusion takes about 30 minutes, with additional preparation and observation.

### Under-the-skin maintenance

The team may select the specific nivolumab formulation given under the skin over about 3–5 minutes. Tell staff about sore, red or bruised skin at the injection site. Dose and route must match the prescription; they are not interchangeable by simple volume.

### Changing maintenance appointments

If the team changes the interval between nivolumab-only doses, they will give you a new appointment plan: the first four-weekly dose is two weeks after the last two-weekly dose, or the first two-weekly dose is four weeks after the last four-weekly dose. Do not change the schedule yourself. This does not change which formulations are permitted for your colorectal treatment.

## Tests and treatment reviews

Checks continue in both phases.

### Blood and thyroid tests

### Scans and markers

The supervising consultant determines imaging, tumour-marker and other disease-monitoring tests. Ask when each test is due and who will review the result.

### Treatment delays

If a dose is held, the team decides whether and when either medicine can restart. Do not assume one medicine is safe to receive when the other has been withheld.

# Symptoms and supportive care

Nivolumab + ipilimumab · colorectal after chemotherapy · NCCP 00551

## Your 21-day cycle.

Timing is a general guide; symptoms may start or stop at other times.

| Treatment or check | Timing in this view |
|---|---|
| **Combination treatment** |  **Day 1** Day 1 in cycles 1–4: nivolumab then ipilimumab |
| **Immune effects** |  **Days 1–21** Any day; also during maintenance or after stopping |
| **Symptoms and tests** |  **Days 1–21** Blood and thyroid tests before each treatment cycle |

This 21-day calendar describes the first four combination cycles. Maintenance uses the separately confirmed two- or four-week schedule.

The next cycle starts 21 days after this one, if your team confirms it.

> Immune effects can happen on any day and after treatment ends. Carry your immunotherapy alert card and tell every healthcare team that you have received nivolumab and ipilimumab.

### Between-treatment notes

### Day 1: Combination visit: cycles 1–4

Blood results and symptoms are reviewed. Nivolumab is given first, then ipilimumab. The team monitors you during treatment and afterwards. Tell the nurse immediately if you become itchy, flushed, breathless, dizzy or suddenly unwell.

### Day 2–21: Between combination visits

Immune inflammation can begin at any time; the calendar cannot predict a safe symptom-free week. After cycle 4, confirm the nivolumab-only dose, route, interval and first maintenance date.

## Support around the medicines

### 1 Sickness prevention and premedication

Nivolumab has minimal and ipilimumab low emetogenic potential. Routine premedication is not usually required; the team individualises anti-sickness treatment and care after any previous reaction.

### 2 Steroids and other treatment for immune effects

Do not self-start or stop steroids. If prescribed, follow the exact taper and any infection-prevention plan. Do not restart cancer treatment until the team explicitly authorises it. If the team changes the interval between nivolumab-only doses, they will give you a new appointment plan: the first four-weekly dose is two weeks after the last two-weekly dose, or the first two-weekly dose is four weeks after the last four-weekly dose. Do not change the schedule yourself. This does not change which formulations are permitted for your colorectal treatment.

### 3 Pregnancy and feeding

Discuss fertility and effective contraception before treatment. Effective contraception continues for at least five months after the last nivolumab dose. Do not breastfeed during treatment; agree any later feeding plan with the team.

## Between visits and practical support

Keep an early-reporting plan.

### Bowels and hydration

Report new diarrhoea, blood in stools or abdominal pain immediately. Immune bowel inflammation can worsen and needs assessment; do not rely on self-treatment alone.

### Energy, hormones and daily life

New marked fatigue, headache, dizziness, weakness, thirst or extra urine can need hormone/glucose tests. Ask for help with eating, activity, sleep, work and caring responsibilities.

### Medicine list and steroids

Tell the team about all medicines, supplements and steroid use. Follow any steroid taper exactly and carry information about hormone replacement.

## Urgent symptoms and care after treatment

Do not wait for the next appointment.

### During infusions

Alert the nurse immediately if you feel itchy, flushed, breathless, dizzy, tight-chested or suddenly unwell.

### Late effects

Keep the alert card and report symptoms after treatment stops. NCCP specifies continuous monitoring for at least five months after the last dose. Some hormone changes need longer follow-up.

## Questions worth asking

Which colorectal setting and MSI-H/dMMR result apply to me?

What benefit and risks apply to this combination?

When are my four combination cycles and first maintenance visit?

Which maintenance dose, route and interval have been prescribed?

Who checks my blood, thyroid and scan results?

Who do I contact urgently during treatment or after it ends?

**Try explaining the plan back:** Which medicines are you having now? What changes after induction? What would make you call today? Ask your team to fill any gaps.

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**Colorectal treatment setting** MSI-H/dMMR metastatic colorectal cancer after fluoropyrimidine-based combination chemotherapy. Other nivolumab/ipilimumab regimens have different doses, routes and timing.

**Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

## Cycles 1–4: nivolumab + ipilimumab every 21 days

**Active medicines:** Nivolumab + Ipilimumab

**Emetogenic risk:** Low combination; nivolumab minimal, ipilimumab low

| Medicine | Dose / route | When |
|---|---|---|
| Ondansetron | 8 mg orally once | Before a combination treatment day, if prescribed under the low-risk antiemetic plan<br>NCIS V8 low-risk reference, distinct from infusion premedication. Review QT risk, electrolytes and interactions. |
| Metoclopramide | 10 mg orally up to three times daily as prescribed | As-needed rescue on subsequent days<br>Confirm contraindications, renal function, interactions and prescribed duration. |

### Additional phase-specific support

- Routine infusion premedication is not usually required. Antiemetics above are a reference requiring individual/local prescription; do not add prophylactic corticosteroids automatically.
- Carry the immunotherapy alert card and report bowel, lung, liver, kidney, hormonal, skin, neurological or cardiac symptoms promptly during and after treatment.
- full blood count (FBC), renal/liver tests, glucose and thyroid function before each cycle; cortisol as clinically indicated. Maintain follow-up for delayed immune toxicity.
- Management uses dose delay or discontinuation, not routine dose reduction. High-dose steroids for immune toxicity require a separately prescribed treatment and taper plan.
- Confirm nivolumab first, separate infusion sets/filters and source-specific observation. The weight-based 00551 regimen and flat-dose 00900 regimen are not interchangeable.
- If the team changes the interval between nivolumab-only doses, they will give you a new appointment plan: the first four-weekly dose is two weeks after the last two-weekly dose, or the first two-weekly dose is four weeks after the last four-weekly dose. Do not change the schedule yourself. This does not change which formulations are permitted for your colorectal treatment.

**Before signing:** verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

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## Cycle 5 onwards: prescribed nivolumab monotherapy schedule

**Active medicines:** Nivolumab

**Emetogenic risk:** Minimal

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

### Additional phase-specific support

- No routine antiemetic prophylaxis or infusion premedication; prescribe symptom relief individually.
- Carry the immunotherapy alert card and report bowel, lung, liver, kidney, hormonal, skin, neurological or cardiac symptoms promptly during and after treatment.
- full blood count (FBC), renal/liver tests, glucose and thyroid function before each cycle; cortisol as clinically indicated. Maintain follow-up for delayed immune toxicity.
- Management uses dose delay or discontinuation, not routine dose reduction. High-dose steroids for immune toxicity require a separately prescribed treatment and taper plan.
- Cycles 1–4: nivolumab 3 mg/kg, then ipilimumab 1 mg/kg, on Day 1 every 21 days. The first nivolumab-only dose is due three weeks after the final combination dose, subject to clinical review. Nivolumab alone: 240 mg intravenous (IV) or 600 mg under the skin every two weeks, or 1200 mg under the skin every four weeks.
- If the team changes the interval between nivolumab-only doses, they will give you a new appointment plan: the first four-weekly dose is two weeks after the last two-weekly dose, or the first two-weekly dose is four weeks after the last four-weekly dose. Do not change the schedule yourself. This does not change which formulations are permitted for your colorectal treatment.

**Before signing:** verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

# What to watch for

You do not need to know which medicine caused a symptom before calling.

### 1 Hormones / nervous system

**Nivolumab + ipilimumab**

New severe headache, dizziness, confusion, weakness, thirst or extra urine needs urgent advice; hormone effects may need long-term treatment.

### 3 Heart

**Nivolumab + ipilimumab**

Chest pain, racing heartbeat or fainting needs urgent assessment.

### 5 Kidneys

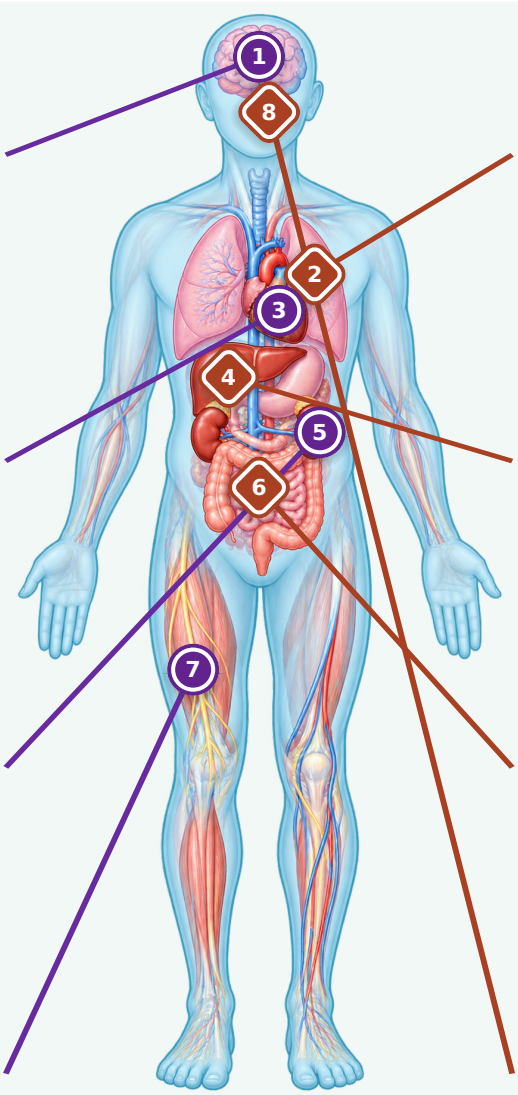
**Nivolumab + ipilimumab**

Report much less urine, new swelling or a change in kidney tests.

### 7 Skin / muscles

**Nivolumab + ipilimumab**

Spreading rash, blisters, severe weakness or muscle pain needs prompt review.




### 2 Lungs

**Nivolumab + ipilimumab**

New cough or breathlessness can signal inflammation or another urgent cause.

### 4 Liver

**Nivolumab + ipilimumab**

Yellow eyes/skin or dark urine may signal inflammation; tests also check the liver.

### 6 Bowel

**Nivolumab + ipilimumab**

New or persistent diarrhoea, blood in stools or severe tummy pain needs urgent advice.

### 8 Eyes / vision

**Nivolumab + ipilimumab**

Report eye pain or new visual change promptly; do not wait for the next cycle.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Contact oncology immediately for new breathlessness, persistent diarrhoea, severe tummy pain, yellow skin or eyes, much less urine, spreading or blistering rash, severe headache, marked weakness, confusion, unusual thirst or passing much more urine. Severe breathing difficulty, chest pain, collapse or seizure needs emergency care. Effects can begin after treatment ends.

**When to get help**

**Call 112 / 999** for severe breathing difficulty, severe chest pain, collapse or a seizure.

**Call oncology immediately:** temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

**Oncology daytime:** 087 12345678 · **Out of hours:** 087 87654321

**EXAMPLE ONLY — team must replace both numbers before patient use.**

SACTCHECK · CHAPTER 04

# Your record and contact card

Nivolumab + ipilimumab · colorectal after chemotherapy · National Cancer Control Programme (NCCP) 00551

Record each cycle separately. Each medicine has its own symptom table. Mark the worst symptoms since your last review, when they occurred and what they stopped you doing. Overlapping symptoms can be recorded under more than one medicine; you do not have to identify the cause.

These are patient-language severity prompts, not a validated questionnaire or a formal Common Terminology Criteria for Adverse Events (CTCAE) grading tool. Your team assigns the clinical grade. Serious symptoms override the columns: call without waiting to decide a severity level.

Use a new record sheet for each cycle or review. Copy or print another sheet when needed; the number of sheets does not prescribe the length of treatment.

If the team changes the interval between nivolumab-only doses, they will give you a new appointment plan: the first four-weekly dose is two weeks after the last two-weekly dose, or the first two-weekly dose is four weeks after the last four-weekly dose. Do not change the schedule yourself. This does not change which formulations are permitted for your colorectal treatment.

Cycle number: _____ Start date: _____ Next review: _____

Record combination cycles 1–4, then the chosen nivolumab-only dose, route and interval. A completed stamp does not approve the next treatment.

**Combination cycle 1 · nivolumab then ipilimumab** · copy this sheet for another cycle of the same phase.

## Nivolumab · Immunotherapy

| Symptom | None | Mild | Moderate / call | Severe / urgent | Emergency signs |
|---|---|---|---|---|---|
| **Breathing / cough** | □ Usual breathing | □ New cough or breathlessness: call promptly | □ Symptoms with usual activity: urgent review | □ Breathless at rest: urgent assessment | □ Severe breathing difficulty / collapse: 112/999 |
| **Diarrhoea** | □ Usual pattern | □ Up to 3 extra stools above normal | □ 4–6 extra stools/ day or at night: call | □ 7+ extra stools/ day or dehydration: urgent review | □ Collapse / severe dehydration: 112/999 |
| **Skin / nail changes** | □ None | □ Mild change: report | □ Pain or daily tasks affected: call | □ Spreading infection or blistering: urgent review | □ Severe illness / collapse: emergency help |
| **Fatigue** | □ Usual energy | □ Tiredness improves with rest | □ Rest does not help; usual tasks affected: call | □ Self-care difficult: urgent review | □ Collapse: 112/999 |

## Ipilimumab · Immunotherapy

| Symptom | None | Mild | Moderate / call | Severe / urgent | Emergency signs |
|---|---|---|---|---|---|
| **Diarrhoea** | □ Usual pattern | □ Up to 3 extra stools above normal | □ 4–6 extra stools/ day or at night: call | □ 7+ extra stools/ day or dehydration: urgent review | □ Collapse / severe dehydration: 112/999 |
| **Skin / nail changes** | □ None | □ Mild change: report | □ Pain or daily tasks affected: call | □ Spreading infection or blistering: urgent review | □ Severe illness / collapse: emergency help |
| **Fatigue** | □ Usual energy | □ Tiredness improves with rest | □ Rest does not help; usual tasks affected: call | □ Self-care difficult: urgent review | □ Collapse: 112/999 |

Dates / doses missed or changed / symptoms affecting daily life / questions:

Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

Cycle number: _____ Start date: _____ Next review: _____

Record combination cycles 1–4, then the chosen nivolumab-only dose, route and interval. A completed stamp does not approve the next treatment.

**Combination cycle 2 · nivolumab then ipilimumab** · copy this sheet for another cycle of the same phase.

## Nivolumab · Immunotherapy

| Symptom | None | Mild | Moderate / call | Severe / urgent | Emergency signs |
|---|---|---|---|---|---|
| **Breathing / cough** | □ Usual breathing | □ New cough or breathlessness: call promptly | □ Symptoms with usual activity: urgent review | □ Breathless at rest: urgent assessment | □ Severe breathing difficulty / collapse: 112/999 |
| **Diarrhoea** | □ Usual pattern | □ Up to 3 extra stools above normal | □ 4–6 extra stools/ day or at night: call | □ 7+ extra stools/ day or dehydration: urgent review | □ Collapse / severe dehydration: 112/999 |
| **Skin / nail changes** | □ None | □ Mild change: report | □ Pain or daily tasks affected: call | □ Spreading infection or blistering: urgent review | □ Severe illness / collapse: emergency help |
| **Fatigue** | □ Usual energy | □ Tiredness improves with rest | □ Rest does not help; usual tasks affected: call | □ Self-care difficult: urgent review | □ Collapse: 112/999 |

## Ipilimumab · Immunotherapy

| Symptom | None | Mild | Moderate / call | Severe / urgent | Emergency signs |
|---|---|---|---|---|---|
| **Diarrhoea** | □ Usual pattern | □ Up to 3 extra stools above normal | □ 4–6 extra stools/ day or at night: call | □ 7+ extra stools/ day or dehydration: urgent review | □ Collapse / severe dehydration: 112/999 |
| **Skin / nail changes** | □ None | □ Mild change: report | □ Pain or daily tasks affected: call | □ Spreading infection or blistering: urgent review | □ Severe illness / collapse: emergency help |
| **Fatigue** | □ Usual energy | □ Tiredness improves with rest | □ Rest does not help; usual tasks affected: call | □ Self-care difficult: urgent review | □ Collapse: 112/999 |

Dates / doses missed or changed / symptoms affecting daily life / questions:

> Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

Cycle number: _____ Start date: _____ Next review: _____

Record combination cycles 1–4, then the chosen nivolumab-only dose, route and interval. A completed stamp does not approve the next treatment.

**Combination cycle 3 · nivolumab then ipilimumab** · copy this sheet for another cycle of the same phase.

## Nivolumab · Immunotherapy

| Symptom | None | Mild | Moderate / call | Severe / urgent | Emergency signs |
|---|---|---|---|---|---|
| **Breathing / cough** | □ Usual breathing | □ New cough or breathlessness: call promptly | □ Symptoms with usual activity: urgent review | □ Breathless at rest: urgent assessment | □ Severe breathing difficulty / collapse: 112/999 |
| **Diarrhoea** | □ Usual pattern | □ Up to 3 extra stools above normal | □ 4–6 extra stools/ day or at night: call | □ 7+ extra stools/ day or dehydration: urgent review | □ Collapse / severe dehydration: 112/999 |
| **Skin / nail changes** | □ None | □ Mild change: report | □ Pain or daily tasks affected: call | □ Spreading infection or blistering: urgent review | □ Severe illness / collapse: emergency help |
| **Fatigue** | □ Usual energy | □ Tiredness improves with rest | □ Rest does not help; usual tasks affected: call | □ Self-care difficult: urgent review | □ Collapse: 112/999 |

## Ipilimumab · Immunotherapy

| Symptom | None | Mild | Moderate / call | Severe / urgent | Emergency signs |
|---|---|---|---|---|---|
| **Diarrhoea** | □ Usual pattern | □ Up to 3 extra stools above normal | □ 4–6 extra stools/ day or at night: call | □ 7+ extra stools/ day or dehydration: urgent review | □ Collapse / severe dehydration: 112/999 |
| **Skin / nail changes** | □ None | □ Mild change: report | □ Pain or daily tasks affected: call | □ Spreading infection or blistering: urgent review | □ Severe illness / collapse: emergency help |
| **Fatigue** | □ Usual energy | □ Tiredness improves with rest | □ Rest does not help; usual tasks affected: call | □ Self-care difficult: urgent review | □ Collapse: 112/999 |

Dates / doses missed or changed / symptoms affecting daily life / questions:

> Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

Cycle number: _____ Start date: _____ Next review: _____

Record combination cycles 1–4, then the chosen nivolumab-only dose, route and interval. A completed stamp does not approve the next treatment.

**Combination cycle 4 · nivolumab then ipilimumab** · copy this sheet for another cycle of the same phase.

## Nivolumab · Immunotherapy

| Symptom | None | Mild | Moderate / call | Severe / urgent | Emergency signs |
|---|---|---|---|---|---|
| **Breathing / cough** | □ Usual breathing | □ New cough or breathlessness: call promptly | □ Symptoms with usual activity: urgent review | □ Breathless at rest: urgent assessment | □ Severe breathing difficulty / collapse: 112/999 |
| **Diarrhoea** | □ Usual pattern | □ Up to 3 extra stools above normal | □ 4–6 extra stools/ day or at night: call | □ 7+ extra stools/ day or dehydration: urgent review | □ Collapse / severe dehydration: 112/999 |
| **Skin / nail changes** | □ None | □ Mild change: report | □ Pain or daily tasks affected: call | □ Spreading infection or blistering: urgent review | □ Severe illness / collapse: emergency help |
| **Fatigue** | □ Usual energy | □ Tiredness improves with rest | □ Rest does not help; usual tasks affected: call | □ Self-care difficult: urgent review | □ Collapse: 112/999 |

## Ipilimumab · Immunotherapy

| Symptom | None | Mild | Moderate / call | Severe / urgent | Emergency signs |
|---|---|---|---|---|---|
| **Diarrhoea** | □ Usual pattern | □ Up to 3 extra stools above normal | □ 4–6 extra stools/ day or at night: call | □ 7+ extra stools/ day or dehydration: urgent review | □ Collapse / severe dehydration: 112/999 |
| **Skin / nail changes** | □ None | □ Mild change: report | □ Pain or daily tasks affected: call | □ Spreading infection or blistering: urgent review | □ Severe illness / collapse: emergency help |
| **Fatigue** | □ Usual energy | □ Tiredness improves with rest | □ Rest does not help; usual tasks affected: call | □ Self-care difficult: urgent review | □ Collapse: 112/999 |

Dates / doses missed or changed / symptoms affecting daily life / questions:

> Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

Cycle number: _____ Start date: _____ Next review: _____

Record combination cycles 1–4, then the chosen nivolumab-only dose, route and interval. A completed stamp does not approve the next treatment.

**Maintenance · confirm nivolumab dose, route and interval** · copy this sheet for another cycle of the same phase.

## Nivolumab · Immunotherapy

| Symptom | None | Mild | Moderate / call | Severe / urgent | Emergency signs |
|---|---|---|---|---|---|
| **Breathing / cough** | □ Usual breathing | □ New cough or breathlessness: call promptly | □ Symptoms with usual activity: urgent review | □ Breathless at rest: urgent assessment | □ Severe breathing difficulty / collapse: 112/999 |
| **Diarrhoea** | □ Usual pattern | □ Up to 3 extra stools above normal | □ 4–6 extra stools/ day or at night: call | □ 7+ extra stools/ day or dehydration: urgent review | □ Collapse / severe dehydration: 112/999 |
| **Skin / nail changes** | □ None | □ Mild change: report | □ Pain or daily tasks affected: call | □ Spreading infection or blistering: urgent review | □ Severe illness / collapse: emergency help |
| **Fatigue** | □ Usual energy | □ Tiredness improves with rest | □ Rest does not help; usual tasks affected: call | □ Self-care difficult: urgent review | □ Collapse: 112/999 |

Dates / doses missed or changed / symptoms affecting daily life / questions:

Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

# Keep your treatment details to hand

**MSI-H/dMMR metastatic colorectal cancer after fluoropyrimidine-based combination chemotherapy** · NCCP 00551 v7a · Document revised 6 November 2026

Print this page **one-sided on A4 at Actual size / 100%**. Cut only the dashed outer border around both panels. **Do not cut the centre line.** Fold with FRONT and BACK facing out, then glue or tape the blank backs together. Finished size: **85.6 × 54 mm**. Keep the stamp side writable.

**FRONT / CONTACTS**

## My Treatment Card

**SACTCheck**

Name: _____

**Nivolumab + ipilimumab · CRC**

4 combination cycles every 21 days, then team-selected nivolumab-only schedule.

Phase: _____ Cycle plan: team to confirm

**Daytime (EXAMPLE)** 087 12345678

**Out of hours (EXAMPLE)** 087 87654321

**Unwell or fever? Call oncology now.**
Severe breathing difficulty, chest pain or collapse: **112 / 999.**

EXAMPLE NUMBERS — replace before patient use.

**BACK / TREATMENT RECORD**

## My cycle record

Record phase and cycle: combination 1–4, then maintenance.

| Cycle | Date / initials |
|---|---|
| | |
| | |
| | |
| | |

My treatment information

Latest treatment date: _____

Plan changed? Ask the team to update this card.

CUT outer border

**FOLD centre — do not cut**

## Keep the record accurate

The team dates and initials each recorded treatment unit when complete. A cycle is complete after its final planned dose; a weekly-treatment stamp records that weekly dose. Delays, omitted doses or a changed plan need a note or a replacement card. A stamp does not approve the next treatment. Keep any medicine-specific alert card too.

## Put your treatment page on your phone

Scan your QR and check the treatment name. Save that page to your home screen.

### iPhone / Safari

Tap Share → Add to Home Screen. Select Open as Web App if offered, then Add.

### Android / Chrome

Tap the three-dot menu → Add to Home screen or Install and create shortcut. Choose the shortcut option and confirm.

Menu wording varies. The QR opens information, not a personal record. Paper stamps do not update the website. Check that the saved icon opens the correct treatment.

## Your local contacts and resources

**Oncology daytime:** 087 12345678 · **Out of hours:** 087 87654321

**EXAMPLE ONLY — team must replace both numbers before patient use.**

Hospital / team: _____

Backup if the team cannot be reached: _____

Additional local services and resources will be added when confirmed. Your team must check and complete this page before patient use.

SACTCheck v0.84.41 · NCCP 00551 v7a · Wallet-card sheet
Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.

# Sources and further information

Nivolumab + ipilimumab · colorectal after chemotherapy · National Cancer Control Programme (NCCP) 00551

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.


1. **National Cancer Control Programme (NCCP) 00551 v7a official regimen**
   https://healthservice.hse.ie/documents/6385/551_Nivolumab_3mgkg_Ipilimumab_1mgkg.pdf
2. **EMA Opdivo product information and patient leaflet**
   https://www.ema.europa.eu/en/documents/product-information/opdivo-epar-product-information_en.pdf
3. **EMA Yervoy product information and patient leaflet**
   https://www.ema.europa.eu/en/documents/product-information/yervoy-epar-product-information_en.pdf
4. **CheckMate 142 four-year combination cohort follow-up**
   https://pubmed.ncbi.nlm.nih.gov/35764271/
5. **HSE treatment safety and urgent advice**
   https://www2.hse.ie/conditions/chemotherapy/side-effects/
6. **NCI: colon cancer treatment — carcinoembryonic antigen (CEA) and follow-up**
   https://www.cancer.gov/types/colorectal/hp/colon-treatment-pdq
7. **NCCP regimen; NCIS Medical Oncology antiemetics V8 Table 1**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf